

Understanding Mental Health Disorders

A documented study guide to early signs, symptoms, diagnosis, treatment and recovery

Conditions covered: Depression • Anxiety Disorders • Bipolar Disorder • Schizophrenia • OCD • Substance Use Disorder • Dementia • Autism Spectrum Disorder • ADHD

Prepared as an educational reference using authoritative public-health and clinical resources. Updated August 2026.

How to Use This Guide

This document is intended to help a reader understand what these conditions are, what early warning signs may look like, how clinicians approach diagnosis, and what evidence-based treatment or support can involve. It is not a diagnostic tool and does not replace evaluation by a qualified healthcare professional.

Important terminology: Several conditions in this guide are treatable rather than simply “curable.” Some are episodic, some are lifelong developmental conditions, and some have many possible underlying causes. Treatment goals therefore differ: symptom reduction, relapse prevention, improved functioning, safety, independence, and quality of life.

A symptom becomes more clinically meaningful when it is persistent or recurrent, represents a change from a person’s usual functioning, causes significant distress or impairment, or occurs as part of a recognizable pattern. A single symptom rarely establishes a diagnosis.

Urgent help is appropriate when someone is at immediate risk of harming themselves or another person, is severely confused or disconnected from reality, cannot care for basic needs, or is experiencing a medical emergency. Emergency services or a local emergency department should be used in such situations.

Quick comparison

Condition	Primary domain	Typical focus of treatment/support
Depression	Mood	Psychotherapy, medication when appropriate, lifestyle/support, relapse prevention
Anxiety disorders	Fear/worry	Psychotherapy, medication when appropriate, exposure/behavioral strategies
Bipolar disorder	Mood episodes	Mood-stabilizing treatment, psychotherapy, sleep/routine management, relapse prevention
Schizophrenia	Psychosis/thought/behavior	Antipsychotic treatment, psychosocial rehabilitation, family/support services
OCD	Obsessions/compulsions	Exposure and response prevention, medication when appropriate
Substance use disorder	Substance-related behavior/health	Behavioral treatment, recovery support, medication for some disorders
Dementia	Cognition/function	Cause-specific care, symptom management, safety, caregiver support
Autism spectrum disorder	Development/social communication	Individualized developmental, educational and functional supports
ADHD	Attention/impulsivity	Behavioral strategies, environmental supports, medication when appropriate

1. Depression

Depression is more than ordinary sadness. Major depressive episodes involve a persistent depressed mood and/or loss of interest or pleasure together with other symptoms that affect functioning.

Core symptoms

- Persistent sadness, emptiness, hopelessness or irritability
- Loss of interest or pleasure in previously enjoyable activities
- Changes in sleep, appetite or weight
- Fatigue or reduced energy
- Difficulty concentrating, remembering or making decisions
- Feelings of worthlessness or excessive guilt
- Moving or speaking unusually slowly, or feeling unusually restless
- Thoughts of death, suicide or self-harm

Possible early warning signs

- Withdrawal from normal activities and relationships
- Noticeable drop in work/study performance
- Neglect of self-care
- Persistent negative thinking or hopelessness
- Sleep/appetite changes that persist rather than being a short-term reaction

How professionals assess it

Diagnosis is clinical. A professional considers symptom pattern, duration, severity, impairment, medical conditions, medications/substances, and possible bipolar-spectrum symptoms before choosing treatment.

Evidence-based treatment / management

- Psychotherapies such as cognitive behavioral therapy and interpersonal approaches
- Antidepressant medication for some people, particularly when symptoms are moderate/severe or psychotherapy alone is insufficient
- Exercise, sleep regularity, social connection and structured daily activity as supportive measures
- More intensive treatments may be considered for severe or treatment-resistant depression

Course and recovery

Depression is highly treatable. Recovery may be gradual, and relapse prevention can include continuing effective treatment, recognizing personal warning signs and maintaining follow-up.

2. Anxiety Disorders

Anxiety disorders involve excessive fear, anxiety or worry that is difficult to control and interferes with daily life. Examples include generalized anxiety disorder, panic disorder, social anxiety disorder and phobias.

Core symptoms

- Persistent excessive worry
- Restlessness or feeling keyed up
- Muscle tension
- Irritability
- Difficulty concentrating
- Sleep disturbance
- Panic attacks or sudden intense fear in panic disorder
- Avoidance of feared situations

Possible early warning signs

- Increasing avoidance of places, people or activities
- Repeated reassurance-seeking
- Constantly anticipating negative outcomes
- Physical tension and sleep disruption
- Difficulty switching off worry even when there is no immediate threat

How professionals assess it

Diagnosis depends on the type of anxiety disorder and considers duration, triggers, avoidance, physical symptoms and functional impact. Medical and substance-related causes may need to be excluded.

Evidence-based treatment / management

- Cognitive behavioral therapy and related evidence-based psychotherapies
- Exposure-based treatment for phobias, panic and some anxiety presentations
- Medication may include certain antidepressants or other clinician-selected options
- Relaxation, exercise, sleep and reducing factors that aggravate anxiety can support treatment

Course and recovery

Many anxiety disorders respond well to treatment. Avoidance can maintain anxiety, so carefully planned therapeutic exposure can be an important part of recovery.

3. Bipolar Disorder

Bipolar disorder involves episodes of depression and periods of mania or hypomania. Mania is substantially more severe than ordinary happiness or high energy.

Core symptoms

- Depressive symptoms: low mood, loss of interest, fatigue, sleep/appetite changes, guilt and hopelessness
- Mania: unusually elevated or irritable mood with markedly increased energy/activity
- Reduced need for sleep
- Rapid speech or racing thoughts
- Inflated confidence or grandiosity
- Increased goal-directed activity
- Impulsive or risky behavior

Possible early warning signs

- Sleeping much less without feeling tired
- Uncharacteristically rapid speech and intense activity
- Sudden major increases in projects, spending, social activity or risk-taking
- Marked change from the person's usual behavior
- Alternation between elevated/activated periods and depressive periods

How professionals assess it

Diagnosis requires looking at the course of symptoms over time, not only the current mood. Clinicians consider family history, substances, medications and medical conditions that can mimic mood symptoms.

Evidence-based treatment / management

- Mood-stabilizing medication and/or other clinician-selected medication
- Psychotherapy and psychoeducation
- Regular sleep and daily routines
- Monitoring early relapse signs and maintaining a treatment plan

Course and recovery

Bipolar disorder can often be managed effectively, but it commonly requires long-term monitoring. Antidepressant treatment should not be started or changed without professional assessment when bipolar disorder is possible.

4. Schizophrenia

Schizophrenia is a serious mental disorder involving disturbances in perception, thinking, emotion and behavior. Psychosis can include hallucinations and delusions.

Core symptoms

- Hallucinations, especially hearing voices
- Delusions or strongly held false beliefs
- Disorganized speech or thinking
- Unusual or disorganized behavior
- Reduced emotional expression
- Social withdrawal
- Reduced motivation or difficulty functioning

Possible early warning signs

- Declining work/school performance
- Social withdrawal and loss of usual interests
- Increasing suspiciousness or unusual interpretations of events
- Sleep disruption
- Difficulty organizing thoughts or communicating clearly
- Decline in self-care

How professionals assess it

Diagnosis is made by a mental-health professional based on symptoms, duration, functional impact and exclusion of other causes such as substances, medications or medical/neurological conditions.

Evidence-based treatment / management

- Antipsychotic medication is a central treatment for psychotic symptoms
- Psychosocial interventions and rehabilitation
- Family education and support
- Supported employment/education and community services
- Hospital treatment may be necessary during severe episodes or when safety is compromised

Course and recovery

Early assessment and treatment can improve outcomes. Treatment is usually ongoing and individualized.

5. Obsessive-Compulsive Disorder (OCD)

OCD involves obsessions (intrusive, unwanted thoughts/images/urges) and/or compulsions (repetitive behaviors or mental acts performed in response to distress or rigid rules).

Core symptoms

- Intrusive unwanted thoughts or images
- Repeated checking, washing, counting or arranging
- Mental rituals or repeated reassurance-seeking
- Fear that something terrible will happen unless a ritual is performed
- Significant time spent on rituals or avoidance

Possible early warning signs

- Increasing time spent checking/cleaning/ritualizing
- Avoiding situations that trigger obsessions
- Repeatedly asking others for reassurance
- Distress when unable to perform a ritual
- Interference with work, study, relationships or daily activities

How professionals assess it

Diagnosis is clinical. Clinicians distinguish OCD from ordinary preferences, worries and repetitive habits by assessing intrusiveness, distress, compulsive behavior, time consumed and impairment.

Evidence-based treatment / management

- Exposure and response prevention (ERP), a form of CBT
- Medication such as an SSRI may be used when appropriate
- Combination treatment may be considered for more severe symptoms
- Family education can help reduce unhelpful reassurance/ritual accommodation

Course and recovery

OCD is treatable. Improvement often requires consistent practice and professional guidance rather than trying to suppress intrusive thoughts.

6. Substance Use Disorder (SUD)

SUD is a pattern of alcohol or drug use that causes clinically significant impairment or distress. It can involve loss of control, craving, continued use despite harm, and difficulty reducing use.

Core symptoms

- Strong cravings
- Using more or for longer than intended
- Repeated unsuccessful efforts to cut down
- Neglecting responsibilities
- Continued use despite physical, psychological or social harm
- Tolerance or withdrawal can occur with some substances

Possible early warning signs

- Increasing frequency or quantity of use
- Using alone or hiding use
- Decline in work, study, finances or relationships
- Repeated risky situations associated with use
- Spending increasing time obtaining, using or recovering from the substance

How professionals assess it

Assessment considers the substance, pattern of use, impairment, withdrawal risk, co-occurring mental/physical conditions and safety. Withdrawal from some substances can be medically dangerous.

Evidence-based treatment / management

- Behavioral therapies and counseling
- Medication-assisted treatment for certain substance use disorders
- Peer/recovery support
- Treatment of co-occurring mental-health conditions
- Medically supervised withdrawal when indicated

Course and recovery

Recovery is possible and can involve multiple treatment attempts. A lapse does not mean treatment has failed. For some substances, stopping abruptly without medical guidance can be dangerous.

7. Dementia

Dementia is an umbrella term for a significant decline in cognitive abilities that interferes with everyday functioning. It has many causes; Alzheimer disease is the most common cause of dementia.

Core symptoms

- Forgetting recent events or repeatedly asking the same questions
- Losing or misplacing things
- Getting lost in familiar places
- Confusion about time or place
- Difficulty solving problems or making decisions
- Difficulty finding words or following conversations
- Difficulty performing familiar tasks
- Changes in mood, personality or social behavior

Possible early warning signs

- Increasing difficulty managing money, medicines or appointments
- Repeated mistakes in familiar activities
- Withdrawal from work or social activities
- Increasing dependence on others for daily tasks
- New personality or behavioral changes

How professionals assess it

Assessment may include history, cognitive testing, physical/neurological examination, medication review and investigations to identify the underlying cause. Some conditions that mimic cognitive decline may be treatable.

Evidence-based treatment / management

- Treat the underlying cause where possible
- Medication for selected symptoms/causes when clinically appropriate
- Cognitive, occupational and functional support
- Home and driving safety assessment
- Caregiver education, respite and social support

Course and recovery

There is no single cure for dementia. Some causes are reversible or partly reversible, while neurodegenerative dementias generally progress over time. Early assessment helps identify treatable contributors and plan support.

8. Autism Spectrum Disorder (ASD)

Autism is a neurological and developmental condition affecting social communication/interaction and patterns of restricted or repetitive behavior or interests. Signs generally begin in early development, although recognition can occur later.

Core symptoms

- Differences in social communication and interaction
- Difficulty interpreting or using nonverbal communication
- Restricted or highly focused interests
- Repetitive movements, speech or behaviors
- Strong preference for routines or difficulty with unexpected change
- Sensory sensitivities or differences

Possible early warning signs

- Developmental differences noticed in early childhood
- Difficulty with reciprocal conversation or social interaction
- Repetitive play or strong routines
- Unusual sensory responses
- Social difficulties becoming more obvious as demands increase

How professionals assess it

Diagnosis is developmental and behavioral. Clinicians consider developmental history, communication, behavior and functioning across settings. There is no single blood test for autism.

Evidence-based treatment / management

- Individualized behavioral/developmental interventions
- Speech-language and occupational support when needed
- Educational accommodations
- Support for co-occurring conditions
- Family education and practical support

Course and recovery

Autism is not something to 'cure' out of a person. The goal is to support communication, independence, participation, well-being and quality of life while respecting individual strengths and needs.

9. Attention-Deficit/Hyperactivity Disorder (ADHD)

ADHD is a neurodevelopmental condition characterized by persistent patterns of inattention and/or hyperactivity-impulsivity that interfere with functioning.

Core symptoms

- Difficulty sustaining attention
- Forgetfulness and losing things
- Poor organization and difficulty completing tasks
- Easily distracted
- Fidgeting or restlessness
- Interrupting or acting before thinking
- Difficulty waiting or regulating impulses

Possible early warning signs

- Repeated missed deadlines or appointments
- Chronic disorganization
- Difficulty starting or finishing routine tasks
- Frequent careless mistakes
- Problems maintaining attention during low-interest tasks
- Impulsivity or restlessness that affects relationships/work

How professionals assess it

Diagnosis requires a comprehensive assessment, including developmental history and symptoms across settings. Sleep problems, anxiety, depression, substance use and other conditions can produce overlapping symptoms.

Evidence-based treatment / management

- Behavioral strategies, coaching and environmental modifications
- Psychotherapy/skills-based interventions when indicated
- Medication may be effective for many people and should be selected/monitored by a clinician
- School/work accommodations when appropriate

Course and recovery

ADHD can persist into adulthood. Good outcomes often come from combining practical systems, environmental changes, skills training and, when appropriate, medication.

When to Seek Professional Help

Consider professional assessment when symptoms persist, repeatedly return, interfere with work/study/relationships, cause significant distress, or represent a clear change from a person's normal functioning.

Examples that warrant prompt assessment include new psychotic symptoms, major mood changes, severe anxiety or compulsions, rapidly worsening memory, significant substance-related impairment, or developmental concerns in a child.

Emergency situations

Seek urgent/emergency help if there is imminent danger of suicide or serious self-harm, violent behavior, severe confusion, inability to care for basic needs, severe intoxication/overdose, or a sudden major change in mental status. Do not rely on this guide for emergency decision-making.

Questions to bring to a clinician

- When did the symptoms first appear?
- How often do they occur and how long do they last?
- What has changed from the person's usual behavior or functioning?
- How are sleep, appetite, work/study and relationships affected?
- Are alcohol, drugs, medications or medical illnesses involved?
- Is there a family history of similar conditions?
- What treatments have already been tried and what happened?

Sources and Further Reading

The following are primary/authoritative public-health resources used as the basis for this educational guide. The linked pages provide downloadable PDFs or detailed evidence-based guidance.

National Institute of Mental Health (NIMH)

NIMH maintains brochures and publications covering depression, anxiety disorders, bipolar disorder, schizophrenia, OCD, ADHD and autism spectrum disorder. The publications include symptoms, diagnosis, treatment and help-seeking information. See the official collection:

<https://www.nimh.nih.gov/health/publications>

NIMH Health Topics: **<https://www.nimh.nih.gov/health/topics>**

NIMH Fact Sheets: **<https://www.nimh.nih.gov/health/publications/fact-sheets>**

World Health Organization (WHO)

WHO Dementia: **<https://www.who.int/news-room/fact-sheets/detail/dementia>** — includes early signs, risk factors, impact and care information.

WHO Risk Reduction of Cognitive Decline and Dementia, 2nd edition (2026):

<https://www.who.int/publications/i/item/9789240123557>

WHO mhGAP Guideline: **https://iris.who.int/bitstream/handle/10665/204132/9789241549417_eng.pdf** — evidence-based guidance for assessment and management of priority mental, neurological and substance-use conditions.

Substance Use Disorder — SAMHSA

SAMHSA treatment resources: **<https://www.samhsa.gov/substance-use/treatment>**

SAMHSA family guide:

<https://library.samhsa.gov/product/what-substance-abuse-treatment-booklet-families/sma14-4126>

NIMH explicitly notes that its public information is educational and that professional help should be sought for individual concerns. This guide follows the same principle.

Medical disclaimer: This document is for education and study only. It does not diagnose any person, prescribe medication, or replace individualized medical/psychological assessment. Treatment decisions should be made with an appropriately qualified healthcare professional.